



Forth Valley Royal Hospital

Ward 3 Mental Health Unit Refurbishment

Pre Construction Information

NHS / SERCO



The Value of Certainty.

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Version Control

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Introduction

This Pre-Construction Information (PCI) document has been prepared by Currie & Brown UK Limited in its role as Principal Designer, in accordance with the requirements of the Construction (Design and Management) Regulations 2015 (CDM 2015). The purpose of this document is to provide designers, contractors and other duty holders with relevant and proportionate health and safety information to enable the safe planning, management and delivery of the proposed works.

The PCI collates known information relating to the site, the scope of works and the associated risks, and is intended to support the early identification, elimination and reduction of foreseeable hazards so far as is reasonably practicable.

The project relates to the refurbishment of Ward 3 within the Mental Health Unit at Forth Valley Royal Hospital. The works are focused on reducing ligature risks and improving patient safety through the installation of ligature-safe doors, windows, furniture and associated internal refurbishments. The works will be undertaken within a live operational healthcare environment, which presents inherent risks relating to infection prevention, patient safety, and continuity of hospital operations.

The information contained within this document is intended to inform the preparation of the Construction Phase Plan together with suitable risk assessments and method statements (RAMS). This document shall be treated as a live document and reviewed as the design and construction progresses.

Notification of project

With reference to Regulation 6 of the CDM Regulations 2015, the works are anticipated to be notifiable subject to confirmation of final programme duration and workforce levels. Where the notification thresholds are met, an F10 notification shall be submitted to the Health and Safety Executive prior to commencement of construction activities.

Where notification is required, the Principal Contractor shall ensure that a copy of the F10 is clearly displayed on site for the duration of the works and kept up to date should any material changes occur. No construction activities shall commence until statutory notification requirements have been satisfied.

Project Details

Description of the Project

The project comprises the refurbishment of Ward 3 within the Mental Health Unit at Forth Valley Royal Hospital, with the primary objective of improving patient safety through the reduction of ligature risks while maintaining full operational continuity of the ward. The works form part of a wider programme of safety improvements within the healthcare environment and are being undertaken in accordance with current healthcare guidance and statutory requirements. The project involves upgrading existing patient accommodation and associated facilities to create a safer, more robust and maintainable environment suitable for a mental health setting.

The scope of works includes the removal and replacement of key building elements within patient bedrooms and en-suite areas, including doors, windows, internal finishes, fixtures and fittings, together with associated electrical, plumbing, door modifications and cleaning required to support the refurbishment. There has been discussion about the communal WC / shower / bathrooms not being part of the project, if this is a change to the scope it will be advised and documented. The works are focused on improving the overall safety, functionality and hygiene of the spaces, while ensuring that the design and installation eliminate or reduce foreseeable risks so far as reasonably practicable. The project also involves coordination of multiple work activities within confined internal areas, requiring careful integration of design, construction sequencing and operational constraints.

All works will be carried out within a live and fully operational mental health ward and will therefore be delivered in a phased manner to minimise disruption to patients, staff and ongoing clinical activities. The works will require suitable barriers/segregation methods to prevent any inadvertent access by anyone other than the construction team. The project requires a high level of coordination with hospital stakeholders to manage access, maintain safe working environments and ensure that infection prevention and control measures are fully implemented throughout the construction period. Consideration must be given to the interaction between construction activities and vulnerable building users, with appropriate segregation, planning and control measures adopted to ensure that safety is maintained at all times.

Site Location and General Environment

The works will be undertaken within Ward 3 of Forth Valley Royal Hospital, located in Larbert, which is a fully operational healthcare facility providing mental health services. The project area is situated within an occupied ward environment, where patient care activities continue throughout the construction period. The ward forms part of a wider hospital setting that includes active clinical departments, staff areas and circulation routes, all of which must remain fully functional during the works.

As the project is being delivered within a live mental health environment, particular consideration must be given to maintaining a safe and secure setting for patients, staff and visitors. The presence of vulnerable service users requires strict control of access, materials and construction activities, alongside robust infection prevention measures. The works will be carefully planned and managed to minimise disruption, control noise and dust, and ensure that the construction activities do not adversely impact the operation of the hospital or the wellbeing of its occupants.

Site Address: Stirling Rd,
Larbert



Surrounding Conditions

The works will be undertaken within an operational hospital environment where surrounding areas remain in continuous use throughout the construction period. Adjacent spaces include active clinical areas, staff circulation routes and support facilities, all of which must remain fully functional. As a result, construction activities must be carefully planned and coordinated to ensure that they do not adversely affect hospital operations or compromise the safety and wellbeing of patients, staff and visitors.

Particular consideration must be given to managing construction related impacts such as noise, dust, vibration and the movement of materials within and around the work area. Temporary segregation measures, including barriers, controlled access routes and clear signage, will be required to separate construction activities from operational areas. The sequencing of works must be aligned with hospital requirements to minimise disruption, maintain safe access and ensure that appropriate infection prevention and control measures are maintained at all times.

Project Timescale

Key Event	Date/Weeks
Anticipated start date	TBC
Anticipated completion date	TBC
Duration	TBC

The Project Team

The project will be delivered by a multidisciplinary team appointed by the Client, Serco, in accordance with the Construction (Design and Management) Regulations 2015 (CDM 2015). Currie & Brown UK Limited has been appointed as Principal Designer and is responsible for planning, managing and monitoring the pre-construction phase, coordinating health and safety matters during the design process, and ensuring that relevant information is communicated to designers and contractors. The Client retains overall responsibility for ensuring that suitable arrangements are in place for managing the project, including the appointment of competent duty holders.

The Principal Contractor will be responsible for planning, managing and coordinating the construction phase, including the preparation and implementation of the Construction Phase Plan, and the development of suitable risk assessments and method statements for the works. The Principal Contractor will manage all contractors and specialist subcontractors involved in the project and ensure that appropriate supervision, communication and control measures are in place. All members of the project team are required to cooperate, coordinate and share relevant information to ensure that the works are carried out safely and without risk to health for those involved and those affected by the works.

The project organisation chart is provided within the appendices for reference.

Ground Conditions

The works are confined to internal refurbishment within an existing hospital building, and no significant external groundworks or deep excavations are anticipated as part of the project. Localised floor preparation works may be required to facilitate installation activities within rooms and en-suite areas; however, these will be limited in extent and depth. Contractors should be aware that the condition of existing substrates and concealed services may vary once exposed, and appropriate precautions must be taken when undertaking any intrusive works to avoid damage to existing building elements or services. Any unforeseen conditions encountered during the works shall be reported to the project team and assessed prior to proceeding.

Extent and location of existing records and plans

Existing drawings, asset information and service records relating to Ward 3 are available; however, the level of detail and accuracy of these records may vary. Information has been developed based on available documentation and site observations, and therefore contractors should not rely solely on existing drawings for construction purposes. All available records should be reviewed prior to the commencement of works, and verification of existing conditions should be undertaken on site where necessary. Any discrepancies, omissions or unforeseen conditions identified during the works must be reported to the project team, and appropriate measures agreed before proceeding.

Client's Considerations, Planning and Management Requirements

Client Brief and Safety Goals

The Client's brief is to deliver the refurbishment of Ward 3 within the Mental Health Unit at Forth Valley Royal Hospital in a manner that enhances patient safety through the reduction of ligature risks, while ensuring that the ward remains operational throughout the construction period. The works are to be carefully planned and executed to minimise disruption to clinical activities and maintain a safe environment for patients, staff and visitors at all times. Particular emphasis is placed on maintaining effective infection prevention and control measures, given the sensitive nature of the healthcare environment and the presence of vulnerable service users. The Principal Contractor to be aware that there may be a number of common areas e.g. WC/bath/shower rooms out with the ensuite in patient bedrooms that present similar risks to those within patient bedrooms.

The project health and safety goals of the Client and the project team are to achieve the following:

- No accidents on site or in areas adjacent to or associated with the construction works.
- No occupational ill health arising from the project.
- No environmental damage resulting from construction activities.
- Minimise disruption to hospital services, staff, patients and visitors.
- Ensure that the construction site is properly secured to prevent unauthorised access at all times.
- Provide safe access and egress to the site and all work areas for those involved in the project.
- Maintain appropriate infection control measures due to the works taking place within a live hospital environment.
- Provide workplaces that are, so far as reasonably practicable, free from risks to the health and safety of those carrying out the works or any others affected by the works.
- Ensure that all contractors and subcontractors comply with the relevant hospital safety procedures and site rules.
- Ensure that risks associated with demolition works, drainage installation, flooring works and equipment handling are properly assessed and controlled.

The Principal Contractor will be required to implement suitable measures to achieve the above objectives in respect of the construction activities under their control. A key objective for all duty holders involved in the project is to cooperate, communicate and coordinate effectively, ensuring that health and safety information is shared and that the risks of injury or incident are minimised. All works must be undertaken in accordance with relevant legislation, healthcare technical guidance and recognised industry best practice.

A robust management structure must be adopted for the project to ensure that the site is managed safely throughout the construction phase. The principles of prevention must be applied and managed so far as reasonably practicable as the project progresses. The Principal Contractor shall provide details of the proposed management structure within the Construction Phase Plan, including arrangements for supervision, communication, coordination and monitoring of health and safety performance during the works.

Communication

Effective communication between all members of the project team will be essential to ensure the safe planning and delivery of the works. The Client, Principal Designer, Principal Contractor and all contractors will be required to cooperate and coordinate their activities in accordance with the requirements of the Construction (Design and Management) Regulations 2015. Regular progress meetings, design coordination meetings and site briefings will be undertaken throughout the project to review progress, discuss health and safety matters and address any emerging risks. Relevant information relating to hazards, design changes and site conditions must be communicated promptly to all parties involved in the works, and appropriate consultation with hospital representatives will be maintained to ensure that construction activities are coordinated with ongoing hospital operations.

Security of the site

The works will be undertaken within an operational mental health ward, and therefore appropriate site security measures must be implemented at all times to protect patients, staff and visitors. The construction area shall be clearly defined and segregated from operational areas using suitable barriers, hoarding and signage, with access restricted to authorised personnel only. The Principal Contractor to ensure appropriate observation of any patient movements near to the works that may impact on patient safety and advise clinical staff of any concerns. The Principal Contractor will be responsible for establishing and maintaining site security arrangements, including controlling access and egress, securing tools, materials and equipment, and ensuring that construction activities do not pose a risk to vulnerable service users. All personnel must comply with the Client's security procedures and site rules, and particular care must be taken to prevent unauthorised access to the work area throughout the duration of the works.

Welfare provision

Suitable welfare facilities will be provided and maintained for all personnel involved in the project in accordance with the requirements of Schedule 2 of the Construction (Design and Management) Regulations 2015. The Principal Contractor will be responsible for ensuring that adequate welfare arrangements are in place prior to the commencement of the works and maintained throughout the construction phase. Welfare provisions shall include access to suitable sanitary conveniences, washing facilities, drinking water, rest areas and facilities for changing and storing clothing where necessary. Where existing hospital facilities are used, this will be subject to prior agreement with the Client, and the Principal Contractor must ensure that the welfare arrangements remain appropriate, hygienic and accessible to the workforce at all times. Please note that it is a client requirement that the welfare facilities provided must be out with the ward environment.

Overlap with the Client's undertaking and other users of the site

The works will be carried out within an operational mental health ward where there will be ongoing interaction between construction activities and the Client's normal operations, including patients, clinical staff, visitors and support services. Particular consideration must be given to ensuring that construction works do not adversely affect patient care or compromise the safety and wellbeing of building users. The Principal Contractor will be required to coordinate closely with the Client and ward staff to manage access, sequencing and working arrangements, including

the use of phased works to minimise disruption. Appropriate segregation, controlled access routes and clear communication will be essential to ensure that construction activities are safely managed alongside ongoing hospital operations.

Client's site rules

All works shall be carried out in strict accordance with the Client's site rules and procedures, which apply to all contractors, subcontractors and visitors involved in the project. The Principal Contractor shall ensure that all personnel receive a site-specific induction prior to commencing work and are made fully aware of hospital requirements, access arrangements, security protocols and emergency procedures. Compliance with designated access routes, personal protective equipment requirements, infection control measures and behavioural standards is mandatory at all times. The Principal Contractor will be responsible for enforcing site rules and ensuring that any breaches are addressed promptly, with works suspended where necessary to prevent risks to health, safety or the continued operation of the hospital.

The Principal Contractor shall be responsible for enforcing the Client's site rules and ensuring that they are embedded within the Construction Phase Plan, risk assessments and method statements. Any breaches of site rules shall be addressed promptly, and corrective action taken where necessary. Works shall be suspended where non-compliance presents a risk to health and safety or to the continued operation of the hospital. The Principal Contractor shall maintain close coordination with the Client to ensure that site rules remain appropriate and are updated or reinforced as required throughout the duration of the works.

Client permit-to-work systems

Certain construction activities will be subject to the Client's permit-to-work systems to ensure that high-risk operations are properly controlled and coordinated with the ongoing operation of the hospital. The Principal Contractor and all subcontractors shall comply with the Client's permit procedures and obtain the necessary approvals prior to commencing any works that may affect existing services, safety systems or the functioning of the facility. This includes activities such as electrical isolation, hot works, works on or near live services, and any operations that may impact fire safety or critical building systems.

The Principal Contractor will be responsible for ensuring that all relevant permits are obtained, understood and adhered to, and that the conditions of each permit are implemented and monitored throughout the duration of the works. No work requiring a permit shall commence until formal authorisation has been issued by the Client or authorised representative. The Principal Contractor to liaise with senior clinical staff to ensure notification of any work that may affect any alarm systems. The Principal Contractor must ensure that any alarm points are protected against accidental activation by site operatives, as this would cause significant risk and distress to patients and medical staff. Should an alarm be activated accidentally. Appropriate supervision, communication and coordination must be maintained at all times to ensure that permit-controlled activities are carried out safely and without risk to patients, staff or the wider hospital environment.

HAI-SCRIBE Requirements and Infection Control Measures

The works are being undertaken within a live operational healthcare environment and therefore strict infection prevention, and control measures shall be implemented throughout all stages of the project. All contractors and subcontractors shall comply with the agreed Stage 3 HAI-SCRIBE procedures and any associated infection control risk assessments applicable to the works.

Construction activities shall be planned and undertaken in accordance with SHFN 30 guidance and all relevant NHS infection prevention protocols. Particular attention shall be given to the control of dust, debris, airborne contaminants and the movement of materials within and around the ward environment.

The Principal Contractor shall ensure that suitable containment measures, segregation arrangements, cleaning procedures and monitoring regimes are implemented and maintained throughout the duration of the works. This may include the use of temporary barriers, sealed work areas, negative pressure systems, controlled access arrangements and enhanced cleaning procedures where required by the Client or infection control team.

All operatives shall participate in and adhere to the agreed HAI-SCRIBE requirements, including any permit systems, inspections, monitoring arrangements and site-specific infection control procedures established for the project. Coordination with hospital staff and the infection prevention team shall be maintained at all times to ensure that construction activities do not adversely affect patients, staff, visitors or ongoing clinical operations.

The Principal Contractor shall ensure that all waste materials are appropriately contained and removed from site in accordance with the agreed infection control procedures. Lockable skips fitted with secure lids shall be used where required to prevent unauthorised access and minimise the spread of dust and contaminants within the hospital environment.

Fire precautions and emergency procedures

The Principal Contractor shall ensure that adequate fire prevention measures and emergency procedures are established and maintained throughout the construction phase. All personnel working on the project must be made aware of the hospital's fire safety procedures, including the location of fire alarm points, fire escape routes and designated assembly points. Particular care must be taken when carrying out activities that may present a fire risk, such as hot works, and such activities must only be undertaken in accordance with the Client's permit-to-work procedures and appropriate control measures.

Emergency procedures must be clearly communicated to all contractors during site induction and reinforced through regular briefings where necessary. In the event of a fire or other emergency, all works must cease immediately and personnel must follow the hospital's emergency arrangements and evacuate the area in a safe and orderly manner. The Principal Contractor will also ensure that suitable fire-fighting equipment is available within the work area and that access to fire escape routes and emergency equipment is kept clear at all times. The Principal Contractor must ensure that all firefighting equipment is located behind secure cover to prevent access by patients or misuse by others.

Emergency Contact Information

In the event of a serious incident, the following emergency services and utilities must be contacted immediately:

Accident & Emergency:

Forth Valley Royal Hospital, Stirling Road, Larbert, FK5 4WR

Tel: 01324 566 000 or 999

Police:

West Bridge Street Police Station, Falkirk, FK1 5AP

Tel: 0141 308 1070 or 999

Please note that clinical staff would call for police attendance if there is any patient incident requiring this response. The Principal Contractor must therefore liaise with clinical staff and ensure communication is in place to ensure that all parties are aware of this situation and any specific control measures required.

Fire & Rescue Service:

Scottish Fire & Rescue Service, Westfield, Falkirk, FK2 9AH

Tel: 01324 629 121 or 999

Gas Emergency: 0800 111 999

Electricity Emergency: 0800 092 9290

These details must be prominently displayed within the site office, welfare units and fire points, and included within the Principal Contractor's Construction Phase Plan.

The Principal Contractor must comply with The Joint Code of Practice: Fire Prevention on Construction Sites, published by the Building Employers Confederation, the Loss Prevention Council and the National Contractors Group. Additionally, the Principal Contractor must ensure compliance with HSG 168 – Fire Safety in Construction, implementing all recommended control measures relating to ignition sources, hot works, temporary fire precautions, safe storage of flammable materials, housekeeping standards, and waste removal. These practices are essential to minimising fire risks on site and ensuring the safety of workers, visitors and the surrounding canal environment.

Site Establishment

The Principal Contractor will be responsible for the establishment and management of the construction site in a manner that ensures the safety of the workforce, hospital staff, patients and visitors. Given that the works are being undertaken within an operational hospital environment, careful planning will be required to ensure that the construction area is clearly segregated from operational areas. Temporary hoarding, barriers and signage will be installed where necessary to define the work zone and restrict access to authorised personnel only.

The site establishment arrangements will also include the provision of suitable areas for the storage of materials, plant and equipment, along with appropriate waste management arrangements. Deliveries and removal of materials must be coordinated with the Client to minimise disruption to hospital operations and ensure that access routes remain safe and unobstructed. The Principal Contractor will also ensure that safe access and egress routes are maintained at all times and that the work area is kept tidy and well organised to reduce the risk of accidents.

HSE Notification

In accordance with the Construction (Design and Management) Regulations 2015, the project will be assessed to determine whether it meets the criteria for notification to the Health and Safety Executive (HSE). Where the project is deemed notifiable, an F10 notification will be submitted to the HSE by the Client or on behalf of the Client prior to the commencement of construction works. The notification will include details of the project, including the Client, Principal Designer, Principal Contractor, project location and the anticipated construction programme.

A copy of the F10 notification must be displayed in a prominent location within the construction site once works commence. The Principal Contractor will ensure that all relevant project information remains accurate and up to date throughout the duration of the works, and any significant changes to the project details will be communicated to the Client and updated within the notification where required.

Smoking Restrictions

Smoking, including the use of e-cigarettes or vaping devices, is strictly prohibited within hospital buildings and in any areas designated as non-smoking by the Client. This restriction applies to all contractors, subcontractors, visitors and personnel involved in the project. Contractors must comply with the hospital's smoking policy at all times and ensure that smoking does not take place within the construction area or in locations that could pose a fire risk or cause nuisance to hospital users.

Where smoking is permitted, it must only take place within designated external smoking areas identified by the Client. The Principal Contractor will ensure that all personnel are made aware of the site smoking restrictions during site induction and that appropriate measures are in place to enforce compliance with the hospital's policies throughout the duration of the works.

Project Health and Safety Hazards

The works associated with the refurbishment of Ward 3 present a range of potential health and safety hazards that must be carefully considered and managed throughout the construction phase. The project involves internal refurbishment activities within a live healthcare environment, including removal and replacement of existing building elements, modification of services, and installation works within confined spaces. These activities may give rise to risks such as manual handling injuries, slips, trips and falls, exposure to noise and vibration, and hazards associated with working near or interfacing with existing building services. Particular attention must also be given to the management of construction-related dust and the safe movement of materials within the ward environment.

In addition to general construction hazards, the project presents significant risks associated with working within an operational mental health facility where vulnerable patients, clinical staff and visitors are present. There is a need to ensure that construction activities do not compromise patient safety, infection prevention and control measures, or the day-to-day operation of the ward. The Principal Contractor will be required to implement robust control measures, including effective segregation of the work area, careful planning of activities, and close coordination with hospital staff. All hazards must be identified, assessed and managed through suitable risk assessments, method statements and site management procedures to ensure that risks are reduced so far as reasonably practicable.

Safety hazards

Boundaries and general access, including temporary access

The construction works will be carried out within Ward 3 of Forth Valley Royal Hospital, and the work area shall be clearly defined and segregated from operational ward spaces at all times. The Principal Contractor will be responsible for establishing suitable site boundaries using temporary barriers, hoarding and clear signage to restrict access to authorised personnel only. Safe access and egress routes must be identified, maintained and coordinated with the Client to avoid conflict with existing hospital circulation routes used by patients, staff and visitors. Any temporary access arrangements required for the movement of personnel, materials and equipment shall be carefully planned, clearly communicated and maintained in a safe condition throughout the duration of the works.

Restrictions on deliveries, vehicular traffic or waste collection or storage

The works will be undertaken within an operational hospital environment where restrictions will apply to deliveries, vehicular movements and the collection and storage of materials and waste. All deliveries to the site must be pre-planned, coordinated with the Client and scheduled to minimise disruption to hospital operations, emergency access routes and staff circulation areas. The Principal Contractor shall ensure that materials and equipment are delivered only at agreed times and via designated access routes, and that unloading activities are managed safely. Storage of materials shall be limited to areas agreed with the Client and maintained in a safe, secure and orderly condition at all times. Construction waste must be managed in accordance with hospital procedures, with prompt removal from the work area to prevent obstruction, reduce fire risk and minimise impact on patients, staff and visitors.

All construction waste skips shall be lockable and fitted with secure lids to prevent unauthorised access, minimise infection control risks and prevent the escape of dust and debris within the live healthcare environment.

Location of existing services

Existing building services are present within and around the work areas and may include electrical, mechanical and plumbing systems associated with the operation of the ward. These services may be concealed within walls, ceilings and floor voids, and their exact locations may not always be fully defined within available records. All existing services shall be treated as live until confirmed otherwise, and contractors must review available information and undertake appropriate verification prior to commencing any intrusive works. Where works interface with existing services, appropriate isolation, lock-off and permit-to-work procedures must be followed. Any uncertainty regarding the presence or condition of services, or any unidentified services encountered during the works, shall be reported immediately to the project team and assessed before work proceeds.

Excavations and Groundworks

No significant external excavations or groundworks are anticipated as part of this project, as the works are confined to internal refurbishment within an existing hospital building. Limited intrusive works may be required within floor constructions to facilitate installation activities, including localised removal of finishes and minor adjustments to existing services. Such activities will be shallow and controlled; however, there remains a risk of encountering concealed services or variations in existing construction. The Principal Contractor shall ensure that appropriate precautions are taken when undertaking any intrusive works, including verification of service locations, use of suitable tools and methods, and implementation of safe systems of work. Any unforeseen conditions encountered during these activities must be reported to the project team and assessed before works continue.

Vibration Impact

Construction activities associated with the refurbishment works, including drilling, cutting and removal of existing building elements, may generate vibration within the structure of the building. Although these works are expected to be localised and of limited intensity, there remains potential for vibration to affect adjacent areas, finishes or sensitive equipment within the operational hospital environment if not properly controlled. The Principal Contractor shall ensure that suitable methods of work and equipment are selected to minimise vibration so far as reasonably practicable, and that works are carefully planned, monitored and coordinated with the Client. Particular consideration must be given to avoiding disturbance to adjacent clinical areas and ensuring that vibration levels do not adversely affect the building structure or ongoing hospital operations.

Hazard considerations

Working at Height

While the majority of the works will be undertaken at ground level, certain activities may require working at height, including installation, removal or adjustment of elements within rooms and service areas. All work at height shall be properly planned, supervised and carried out by competent personnel using suitable access equipment. The Principal Contractor shall ensure that appropriate control measures are implemented in accordance with the Work at Height Regulations 2005, including the selection of appropriate equipment, prevention of falls, and protection of those working below. All equipment must be inspected and maintained in a safe condition, and safe systems of work must be followed at all times.

Lifting operations

Lifting operations may be required during the project for the movement, removal and installation of materials, equipment and building components within the ward environment. These activities may involve both manual handling and the use of mechanical lifting aids and must be carefully planned and carried out by competent personnel to minimise risk. The Principal Contractor shall ensure that all lifting operations are undertaken in accordance with the Lifting Operations and Lifting Equipment Regulations 1998 (LOLER) and the Provision and Use of Work Equipment Regulations 1998 (PUWER), with appropriate risk assessments, supervision and control measures in place. Adequate space and safe access must be maintained to allow lifting activities to be carried out without risk to workers, patients, staff or visitors.

Live Services

Existing live services are present within the work areas and may include electrical, mechanical and plumbing systems essential to the operation of the ward. These services shall be assumed to be live at all times unless confirmed otherwise, and they present a significant risk during refurbishment activities. The Principal Contractor shall ensure that all works in proximity to existing services are properly planned and controlled, including the identification, isolation and protection of services where required. Appropriate permit-to-work procedures, lock-off arrangements and verification processes must be implemented prior to commencing any works that may affect live systems. Any uncertainty regarding the presence, condition or location of services shall be reported to the project team, and works shall not proceed until it is safe to do so.

Slips /Trips & Falls

There is a potential risk of slips, trips and falls during the construction works due to the presence of materials, tools, debris and temporary changes to floor conditions within the work areas. Activities such as removal of existing elements, installation works and movement of materials may create uneven surfaces or temporary obstructions, increasing the likelihood of incidents if not properly managed. The Principal Contractor shall implement suitable control measures, including maintaining good housekeeping, keeping access routes clear, providing adequate lighting

and ensuring that any hazards are clearly identified and protected. All personnel must remain vigilant and follow safe working practices to minimise the risk of slips, trips and falls.

Hand-arm vibration

Construction activities associated with the works may involve the use of powered tools and equipment that can expose operatives to hand-arm vibration. Prolonged or excessive exposure may result in health conditions such as Hand-Arm Vibration Syndrome (HAVS) if not properly controlled. The Principal Contractor shall ensure that risks associated with vibration are assessed and managed in accordance with the Control of Vibration at Work Regulations 2005. Appropriate control measures shall include the selection of low-vibration equipment where practicable, limiting exposure duration, implementing task rotation and ensuring that tools are properly maintained. Operatives shall be provided with suitable information, instruction and training, and exposure levels shall be monitored to ensure they remain within acceptable limits.

Construction Related Dust & Silica Dust Particles

Construction activities associated with the works, including drilling, cutting and removal of existing building elements, may generate dust, including respirable crystalline silica (RCS), which can present significant health risks if inhaled. The Principal Contractor shall ensure that exposure to construction dust is adequately controlled in accordance with relevant regulations and best practice. Suitable control measures shall include the use of dust suppression techniques, local extraction systems, effective segregation of the work area and appropriate cleaning regimes to prevent the spread of dust to surrounding areas. Operatives shall be provided with suitable respiratory protective equipment where required, and particular care must be taken to protect patients, staff and visitors within the operational healthcare environment from exposure.

Musculoskeletal Disorders.

Construction activities associated with the works may involve tasks that could contribute to musculoskeletal disorders (MSDs), including repetitive movements, awkward postures and the handling of materials and equipment within confined spaces. Such activities may place strain on the body if not properly managed. The Principal Contractor shall ensure that risks associated with musculoskeletal disorders are assessed and controlled through appropriate planning of tasks, use of mechanical aids where practicable, and implementation of safe working methods. Operatives shall be provided with suitable information, instruction and training to promote good ergonomic practices and reduce the risk of injury.

Manual Handling

Manual handling activities are likely to be required during the works, including the movement of materials, equipment and components within the ward environment. These activities may involve lifting, carrying, pushing or pulling loads, which could present a risk of injury if not properly controlled. The Principal Contractor shall ensure that manual handling risks are assessed and managed in accordance with the Manual Handling Operations Regulations 1992 (as amended). Control measures shall include reducing the need for manual handling where practicable, using

mechanical aids, ensuring loads are manageable and handled by sufficient personnel, and providing appropriate training in safe lifting techniques. All operatives must follow safe working practices to minimise the risk of injury.

Noise from Construction Activities

Construction activities associated with the works may generate noise from the use of tools, equipment and installation processes, which could pose a risk to operatives and cause disturbance within the operational hospital environment. Prolonged exposure to elevated noise levels may impact hearing, and uncontrolled noise may affect patients, staff and ongoing clinical activities. The Principal Contractor shall ensure that noise risks are assessed and managed in accordance with the Control of Noise at Work Regulations 2005. Suitable control measures shall include the selection of low-noise equipment where practicable, limiting the duration of noisy activities, maintaining plant in good condition and scheduling works to minimise disruption. Operatives shall be provided with appropriate hearing protection where required, and noise levels shall be controlled to reduce impact on the surrounding environment.

Buried Services and Excavations

No external excavation works are anticipated as part of this project; however, limited internal intrusive works may be required within floor constructions, where concealed services could be present. Existing services may run within or beneath floors and their exact locations may not be fully defined within available records. The Principal Contractor shall ensure that all available information is reviewed, and appropriate checks are undertaken prior to commencing any intrusive activities. Any services encountered shall be treated as live until confirmed otherwise, and suitable precautions must be implemented to prevent damage or injury. Where required, services shall be isolated in accordance with permit-to-work procedures, and any unidentified services or unexpected conditions must be reported to the project team and assessed before works continue.

Further hazard considerations

In addition to the hazards identified above, consideration must be given to the risks associated with undertaking construction works within an operational mental health environment. The presence of patients, staff and visitors in surrounding areas requires that all activities are carefully planned and controlled to prevent risks arising from noise, dust, movement of materials and interaction between construction personnel and building users. Particular attention shall be given to maintaining effective segregation of the work area, managing access routes and ensuring that construction activities do not interfere with clinical operations. The Principal Contractor shall ensure that all foreseeable hazards are identified, assessed and controlled through appropriate risk assessments, method statements and site management procedures, and that any new or unforeseen hazards are promptly communicated and managed to maintain a safe working environment.

Health hazards

Asbestos, including results of surveys

Based on available information, no asbestos-containing materials are expected to be present within the areas affected by the proposed works. The building in which the works are to be carried out was constructed after the introduction of legislation prohibiting the use of asbestos-containing materials in construction; however, contractors shall not assume that the building is entirely free from asbestos. All available asbestos survey information must be reviewed prior to the commencement of works, and appropriate control measures shall be implemented in accordance with the Control of Asbestos Regulations 2012. If any suspect materials are encountered during the works, all activities in the affected area shall cease immediately and the material shall be reported to the Client or responsible person for further assessment. No work shall recommence until the material has been properly inspected and confirmed safe, and any required remedial actions have been completed.

Legionella Risks from Water Systems

The works will involve activities within areas where existing water systems are present, and there is a potential risk of Legionella growth if systems are not properly managed during the construction period. Temporary isolation, modification or reduced use of water services may create conditions for stagnation, which can increase the risk of bacterial growth. The Principal Contractor shall coordinate with the Client and relevant facilities management teams to ensure that water systems are managed in accordance with applicable guidance and site-specific procedures. Appropriate control measures shall include minimising periods of stagnation, implement flushing and disinfection regimes where required, and ensuring that systems are tested and safely reinstated prior to use. All works affecting water systems must be carefully planned and carried out to prevent risks to health for patients, staff and contractors.

Existing storage of hazardous materials

The works will be undertaken within an operational healthcare environment where hazardous materials may be present as part of normal hospital activities, including cleaning agents, disinfectants and other maintenance-related substances. These materials may pose a risk if not properly identified and managed during the construction works. The Principal Contractor shall coordinate with the Client and relevant hospital personnel to identify any hazardous materials within or adjacent to the work areas prior to commencement. Where necessary, such materials shall be removed, isolated or adequately protected to prevent exposure or interference with construction activities. All substances used during the works shall be managed in accordance with the Control of Substances Hazardous to Health (COSHH) Regulations, with appropriate handling, storage and disposal procedures implemented.

Existing structures containing hazardous materials

Based on available information, no existing structures containing hazardous materials are expected to be present within the areas affected by the proposed works. However, given the nature of refurbishment activities, there remains a potential for previously unidentified materials to be encountered during intrusive works. The Principal Contractor shall ensure that all operatives remain vigilant when undertaking demolition or installation activities, and any suspect

materials shall be treated with caution. If any materials are identified that may present a hazard, works in the affected area shall cease immediately and the issue shall be reported to the Client or project team for further assessment. No works shall recommence until the material has been properly assessed and deemed safe to proceed.

Hazard management/key hazards

Hazard management

Effective hazard management is essential to the safe delivery of the works and shall be carried out in accordance with the general principles of prevention set out in the Construction (Design and Management) Regulations 2015. Known hazards associated with the project have been identified during the design and pre-construction stages, and where reasonably practicable, risks have been eliminated or reduced through design decisions, planning and specification. However, residual risks remain due to the nature of the works and the operational healthcare environment, and these must be appropriately managed throughout the construction phase.

The Principal Contractor shall be responsible for implementing robust arrangements for managing health and safety risks through the development and maintenance of the Construction Phase Plan, supported by suitable and sufficient risk assessments and method statements. Control measures shall be clearly communicated to all personnel and monitored throughout the duration of the works. Where changes occur to the scope, sequencing or site conditions, hazard management arrangements shall be reviewed and updated accordingly. Any new or unforeseen hazards shall be promptly reported, assessed and managed in coordination with the Client and Principal Designer to ensure that risks are reduced so far as reasonably practicable.

Design Hazard and Risk information

During the design stage of the project, consideration has been given to the identification and management of potential hazards associated with the proposed works, in accordance with the requirements of the Construction (Design and Management) Regulations 2015. Designers have sought to eliminate hazards where reasonably practicable and reduce risks through design decisions, planning of construction activities and coordination of interfaces within the existing building. Particular attention has been given to the challenges of working within a live healthcare environment, including the need to minimise disruption, maintain safe access and ensure that risks to patients, staff and visitors are appropriately addressed.

Despite these measures, certain risks cannot be entirely eliminated through design and will remain during the construction phase. These include risks associated with working in confined internal spaces, interaction with existing services, and the management of construction activities within an operational ward. The Principal Contractor shall be responsible for ensuring that these risks are further assessed and controlled through appropriate risk assessments, method statements and safe systems of work prior to commencement of the relevant activities.

Residual Design Hazard and Risk information

Despite the efforts made during the design stage to eliminate or reduce risks where reasonably practicable, certain residual hazards remain due to the nature of the works and the constraints of the existing environment. These residual risks arise primarily from undertaking refurbishment activities within a live mental health ward, including interaction

with existing building services, working within confined internal spaces and the need to coordinate construction activities alongside ongoing clinical operations. Additional considerations include the movement of materials, potential disturbance to surrounding areas and the management of construction activities in proximity to vulnerable service users.

The Principal Contractor shall ensure that all residual risks are adequately assessed and managed through the development of suitable risk assessments, method statements and safe systems of work prior to the commencement of the relevant activities. Particular attention shall be given to maintaining effective segregation of the work area, controlling access, managing infection prevention measures and ensuring that all personnel are aware of the risks associated with working within an operational healthcare environment. These risks must be clearly communicated to all contractors and monitored throughout the construction phase to ensure that they are controlled so far as reasonably practicable.

Arrangements for Coordination of On-Going Design Work

Design coordination will be undertaken throughout the project to ensure that health and safety risks continue to be effectively managed as the design develops. The Principal Designer shall coordinate all design-related matters, including any changes that may arise during the project, and ensure that relevant information is communicated to all duty holders. Ongoing collaboration between the Client, designers and the Principal Contractor will be required to ensure that design decisions are aligned with construction methodologies and site constraints, particularly within the operational healthcare environment.

The Principal Contractor shall notify the Principal Designer of any proposed changes to the scope of works, construction methods or temporary works that may impact health and safety prior to implementation. No design-related changes shall be undertaken without appropriate review and agreement. Any updated design information or identified risks shall be communicated to all relevant parties and incorporated into the Construction Phase Plan and associated risk assessments, ensuring that coordination is maintained and risks are managed throughout the duration of the works.

Unforeseen Risks Arising During the Construction Works

Despite careful planning and the identification of known hazards during the design and pre-construction stages, unforeseen risks may arise during the course of the construction works. These may include the discovery of previously unidentified services, unexpected conditions within the existing structure or variations in existing building elements that were not evident from available records or surveys. Such circumstances may introduce additional health and safety risks that require immediate consideration and appropriate control measures.

The Principal Contractor shall ensure that, where any unforeseen hazards or conditions are encountered, the relevant works are stopped, and the issue is reported to the Client and Principal Designer without delay. The situation shall be assessed and suitable control measures agreed before works are permitted to continue. Where necessary, the design may require review or amendment to address newly identified risks. Effective communication and coordination between all members of the project team will be essential to ensure that unforeseen risks are managed safely and appropriately.

Construction Materials

Construction materials used in the project shall be selected to ensure suitability for a healthcare environment and compliance with relevant standards and guidance. Materials shall be appropriate for use within a clinical setting, taking into account durability, hygiene and safety requirements. Consideration shall be given to ensuring that materials do not introduce additional risks to health, particularly within an operational ward environment where patients and staff may be affected by construction activities.

The Principal Contractor shall ensure that all materials are handled, stored and installed in accordance with manufacturer instructions and relevant safety procedures. Any substances that may present a risk to health shall be assessed and managed in accordance with the Control of Substances Hazardous to Health (COSHH) Regulations. Appropriate control measures, including safe storage, handling and disposal practices, shall be implemented to prevent risks to operatives, patients, staff and visitors throughout the construction phase.

Method Statements

All work sequences must be risk assessed by the contractor carrying out the works. Where these work sequences are found to contain risks or hazards which require specific control measures to be put in place and adopted, a Statement of Work/Method Statement will be required to be formulated by the contractor in order to control that risk or hazard. Statement of Work/Method Statement for the project must include the following site-specific information:

1. The item of work
2. The location, including access and site boundary
3. Duration of the works, including key stages
4. Safety arrangements required
5. Personnel involved – numbers, skills, training and PPE requirements
6. Briefing arrangements for site personnel affected by the Method Statement
7. Resources to be used (plant and machinery)
8. Detail of how the work will be carried out.
9. Detail of temporary works required
10. Risks identified with proposed method of control
11. Emergency arrangements – fire, injury etc.

Management of Significant Hazards

The management of significant hazards associated with the project will be the responsibility of the Principal Contractor, who shall ensure that appropriate systems and control measures are implemented throughout the construction phase. The Principal Contractor shall develop and maintain a Construction Phase Plan that clearly identifies significant hazards associated with the works and outlines the arrangements for managing and controlling these risks. Suitable and sufficient risk assessments and method statements shall be prepared for all high-risk activities and communicated to all personnel prior to the commencement of work.

Significant hazards may include activities such as working within a live healthcare environment, interaction with existing services, manual handling of materials and equipment, exposure to noise and dust, and the movement of personnel and materials within confined spaces. The Principal Contractor shall ensure that these hazards are effectively managed through appropriate supervision, training, safe systems of work and ongoing monitoring of site activities. Effective communication and coordination with the Client and project team shall be maintained at all times to ensure that risks are controlled and that the health and safety of workers, patients, staff and visitors is protected throughout the duration of the works

The Health and Safety File

The Health and Safety File

It is the responsibility of the Principal Contractor to provide sufficient information for the Health and Safety File as required by the Construction (Design and Management) Regulations 2015.

The Health and Safety File is to be compiled by the Principal Designer and shall be based on the following:

Key Health and Safety File information shall include the following:

- A brief description of the work carried out.
- Any hazards that have not been eliminated through the design and construction process, and how they have been addressed i.e. Residual Hazards.
- Key structural principles (e.g., bracing, sources of substantial stored energy including pre or post-tensioned members) and safe working loads for floors and roofs.
- Hazardous materials used
- Information regarding removal or dismantling of plant and equipment (e.g., any special arrangements for lifting such equipment).
- Health and safety information about equipment provided for cleaning or maintaining the structure.
- The nature, location, and markings of significant services, including underground cables; gas supply equipment; fire-fighting services, etc
- Fire strategy.
- Information and as-built drawings of the building, its plant and equipment
- Cleaning access and maintenance strategy.
- Plant maintenance strategy.

Health and Safety File information shall be provided by the Principal Contractor separately to completion handover documentation.

The Health and Safety File and O&M Manuals in the following formats unless otherwise agreed by the Client:

- 1 x hard copy version in Arial font size 12, including A3 colour drawings
- 1 x electronic copy issued via WeTransfer
- 1 x USB flash drive containing all documentation

All electronic information shall include drawings in both DWG and PDF formats.

Appendices

Appendix B – Work Involving Particular Risks

Schedule 3 of the Construction (Design and Management) Regulations 2015 lists significant hazards that require specific measures to be taken by the Principal Contractor. See Table 1. Further project specific significant hazards are listed in Table 2.

Table 1

No.	Activity	Comment / Note
1.	Refurbishment works within a live healthcare environment	Works to be carefully managed to minimise disruption to patients, staff and visitors, with strict segregation and coordination required.
2.	Infection control risks (HAI-SCRIBE)	Construction activities may generate dust and contaminants; strict infection control measures, containment and cleaning regimes required.
3.	Patient safety and behavioural risks	Presence of vulnerable service users may introduce risks from interaction with construction activities; strict supervision, segregation and control of materials and tools required.
4.	Work in proximity to existing services	Electrical, plumbing and other services may be present and must be identified, isolated and protected where required.
5.	Construction activities generating dust	Dust from drilling, cutting and removal works must be controlled to prevent exposure and contamination within the ward.
6.	Manual handling of materials and equipment	Movement of materials within confined spaces may present handling risks and requires appropriate controls and use of aids.
7.	Working at height	Activities involving access to elevated areas must be properly planned and carried out using suitable equipment.
8.	Use of tools and equipment	Powered tools may introduce risks associated with noise, vibration and moving parts and must be used by competent personnel.
9.	Noise from construction activities	Construction noise may impact hospital operations and must be controlled and managed appropriately.
10.	Slips, trips and falls	Temporary changes to floor conditions and presence of materials may create hazards within the work area.
11.	Work in confined internal spaces	Limited working areas require careful planning to ensure safe access, egress and coordination of activities.

Appendix C - Construction Phase Plan Criteria

Section 1 - General Project Information
1.1 Description of the project
1.2 Programme details
1.3 Details of Client, PD, Designers, PC and other consultants
1.4 The management structure and responsibilities
Section 2 – Management Arrangements
2.1 Project health and safety aims and goals
2.2 The site rules
2.3 Arrangements to ensure cooperation between project team members and coordination of their work, e.g. regular site meetings
2.4 Arrangements for involving workers - consultation with workforce
2.5 Site induction / Site Training
2.6 Welfare facilities
2.7 Fire and emergency procedures (Fire Plan)
2.8 Security arrangements
2.9 First aid arrangements
2.10 Accident/incident reporting and investigating, RIDDOR
2.11 Monitor and review health and safety performance
2.12 Site plan/traffic management plan
2.13 Significant safety risks – reference Schedule 3, CDM 2015
2.14 Health and Safety File Information

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